

العمر(تاريخ الميلاد) : 30-04-1968 المهنة : ربه منزل

----- التعليق على التشخيص :

رئيس
المجلس الطبي المتخصص

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Name: اميمه عبد الرحيم محمد عبد الرحمن National ID: 26804300100686	
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تقرير طبي

السيد
تحية طبية وبعد,,

إيماء للطلب المقدم من	بشأن تقرير طبي للمريض
بقسم:	رقم طبي:

نتشرف بان نوضح فيما يلي تقريرنا عن حالته
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تاريخ وساعة الدخول : -- --:-- yyyy-mm-dd	تاريخ وساعة الخروج : PM 01:17 05-07-2026
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وبعد الكشف والفحوصات والمتابعة وجد انه
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المریضة تعاني من ورم غير حميد بالثدى و تحتاج الي Second Line Hormonal Treatment Aromatase Inhibitor
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التاريخ : 05-07-2026	الوقت : -- --:--
	الطبيب المعالج : د/شيماء لاشين

جمهورية مصر العربية
بطالة عميق الشخصية



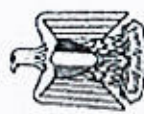
العمه
عبد الرحيم محمد عبد الرحمن
٢ من الشرفه زواقي العرب
الوزاقي - الجيزه



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٢٦٨٠٤٣٠٠١٠٠٢٨٦
روية منزل
أش
مستلمة
مترجمة
حدى خلفى ابورواش
٢٠٢٧/٠٣/١٧
البطاقة سارية حتى





Name: اميمة عبدالرحيم محمد

Age: 53 years.

Clinical data: history of CBS, & axillary node dissection (IDC with infiltrated margin)

Specimen type: left simple mastectomy

Date (Received): 25/4/2021

Date (reported): 4/5/2021

Lab. Number: 895 /21

Hospital Number: 744/21 اقتصادي

Gross Description:

Left simple mastectomy measured 25x11x2cm, covering skin ellipse 22x11cm including nipple and areola and showed a scar 4 cm long located para-areolar, cut section of the breast revealed a diffuse greyish white areas admixed with fat.

Microscopic examination:

Revealed breast tissue with maintained lobular architecture, adenosis and fibrosis with sclerosing lesion more cellular in the center with distorted and compressed ductules, peripherally has more open or dilated ductules, no evident tumor tissue, free nipple and areola, free margin.

Diagnosis:

Known case of IDC (previous CBS), simple mastectomy:

- No remnant of tumor tissue
- Area of sclerosing adenosis
- Free nipple & areola
- Free deep margin

Prof. Dr. Gamal Abd EL Aty

Dr. Rana Fathy

Name: أميمة عبد الرحيم محمد عبد الرحمن Age: years.

Clinical data: breast cancer

Specimen type: left CBS, axillary clearance and lateral group of axillary LNs

Date (Received): 28/3/2021

Date (reported): 4/4/2021

Lab. Number: 644/21

Hospital Number: 744/21

Gross Description:

Three specimens were received:

- 1) A fibrofatty tissue piece measuring 8.5x6.5x3.5cm, cut sections revealed an irregular firm gray white mass measuring 2.3x1.5cm located just below the skin and away from superior (least) margin by 0.1cm.
- 2) fibrofatty tissue piece (Axilla) measuring 9.5x5cm entangling 20 L.N up to 1.5cm.
- 3) fibrofatty tissue piece (lateral group of axilla) measuring 7.5x3cm entangling 4 L.N up to 2cm.

Microscopic Examination:

Reveals an infiltrating malignant epithelial growth. The tumour is made up of groups and small sheets of pleomorphic hyperchromatic malignant ductal epithelial cells. Stromal desmoplasia is moderate with areas of necrosis. Cellular and nuclear atypia are moderate [grade II]. Minor intraduct component is detected. Superior margin was positive to tumor tissue. Other margins and skin were free. Axillary lymph nodes showed deposits in 3 nodules out of 20 nodules, the lateral group of axillary lymph nodes were free 0/4.

Diagnosis:

Breast cancer, left CBS, and axillary clearance:

- Invasive duct carcinoma, grade II with minor in situ component.
- Superior margin was involved, other margins were free.
- Free skin.
- Total nodal count 3/24
- Stage pT2 pN1.

Prof. Dr. Gamal Abd El Aty.
Dr. Rana Fathy

مركز ابحاث داء السل
مركز ابحاث داء السل

Name : Omayma Abdelrahman Mohamed Date : 6/3/20

**POST CONTRAST MULTI-SLICE CT SCAN OF THE CHEST,
ABDOMEN & PELVIS REVEALED: -**

Known case of breast cancer.

1. Chest:

- Left breast mass is seen measuring about 2 x 1.8 cm with pathologically enlarged left axillary lymph nodes the largest is seen measuring about 1.2 x 1 cm.
- No sizable pulmonary nodules, masses, areas of consolidation or cavitary lesions.
- No hilar or mediastinal lymph nodes.
- No evidence of pleural effusion, pleural thickening or masses seen.
- Normal cardiac size. No pericardial effusion detected.
- Normal CT appearance of the thoracic aorta and the great vessels.

2. Abdomen and pelvis:

- The liver is enlarged in size with homogenous low CT texture. No focal lesions or contour irregularities.
- Patent homogeneously enhancing portal vein and its main branches as well as its main tributaries.
- No intra-hepatic or extra-hepatic biliary radicles dilatation.
- Normal size & parenchymal texture of the spleen with no focal lesions seen.
- Normal size, parenchymal thickness & density of both kidneys with no stones or back-pressure changes.
- Normal appearance of the pancreas, aorta, I.V.C and both suprarenals.
- Smooth filling of the urinary bladder with no filling defects or diverticular outpouching.
- The uterus is bulky in size.
- Normal appearance of the other pelvic organs.
- No abdominal or pelvic collections.
- No lytic or sclerotic bone lesions.

OPINION:

- ***Known case of breast cancer showing left breast mass and left axillary lymph nodes.***
- ***No hepatic, osseous or pulmonary deposits.***

Dr. Ahmed Rozek, MSc

Dr. Amr Farouk, MD